

Routine Visit Scheduling

A burst of scheduler work at admission, then the clinician runs their own week

TRIGGER

Admission
SOC or eval visit

PHASE 1 · FREQUENCY PLOTTING & INITIAL ASSIGNMENT – the burst

× N disciplines

Evaluating clinician
plots own discipline
frequency

× N disciplines

RN also plots aide
frequency + MSW / ST
initial eval

Each submission
generates its own
assignment task

Scheduler assigns
visits to care team

RN at the SOC; PT and OT at their own evals, 1–2 days later

485 SUBMITTED – after this, every order is an add-on

add-on orders route through DCS approval and auth before reaching the scheduler

PHASE 2 · STEADY STATE – the clinician runs their own week

NO SCHEDULER WORKFLOW

Evaluate own
capacity for
the week

- Points already committed
- Days off · PTO · on-call
- Documentation still owed

Prioritise clinical
need across
the caseload

- Who is unstable
- Wound · IV · catheter due
- Labs due
- Who can safely wait

Group visits
geographically

- Who sits near whom
- Drive time, not distance
- Bridges · rivers · crossings
- Where the day starts

Test against hard
constraints

- Wound care timing
- Catheter and IV schedules
- Caregiver must be present
- Dialysis · MD appointments

Confirm with the
patient — day before

- “Can you come later?”
- “Not Mondays”
- Patient not home
- → see the panel below

Route — HCHB suggests,
clinician adjusts

- The suggested route
- Patient time windows
- Traffic and time of day
- Where the day must end

what the clinician is working around at each step

THE DAY-BEFORE NEGOTIATION – what the clinician has to hold

HARD – accept it and build the day around it

- Dialysis days and times
- MD and specialist appointments
- Caregiver's working hours
- Patient genuinely not home

SOFT – negotiable, and worth holding the line on

- “Can you come after lunch?”
- “Not first thing” · “Not Mondays”
- A preferred time with no reason behind it

The first visit at 8 or 9am is the single largest lever on an individual clinician's capacity.

Newer clinicians let the patient set the time, become over-accommodating, and push the cost onto the rest of the team.

AT THE VISIT · THE CLINICIAN'S FIVE DISPOSITIONS

Accept
visit delivered

Reschedule
within the week

Reassign
back to scheduler —
usually RN to her own LPN

the only recurring scheduler trigger

Miss
→ compliance chain

Decline
back to scheduler —
must go to a different clinician

the scheduler sees it was declined

BOUNDARIES

OASIS visits do not move

Medicare week is Sunday–Saturday

Inside the 60-day certification period

Auth still gates assignment

WHERE IT
BREAKS

RN cannot reassign to her own LPN
an HCHB rule, not Medicare

Supervisor cannot see
the supervisee's schedule

Day-before calls are
unpaid evening work

The weekly logic is
undocumented and unassisted